DR Care Solutions

Danielle Robertson Consulting Pty Ltd trading as DR Care Solutions

ABN 44 605 748 392 ACN 605 748 392
3408/1 Sergeants Lane, St Leonards NSW 2065

Life Plan™

This Life Plan™ is completed for all care needs assessments undertaken by a consultant from DR Care Solutions. It's used as a guideline as to the care options available for a specific client(s). It is a central place to capture your preferences around future ageing related care, types of support and living arrangements.

CLIENTS NAME: **MIMI MARIA PRINS**
DOB: **03.12.1947 (77 years of age)**
CLIENT ID: **DRCS 0522**

Issues covered:	**Background and relevant history;** Angela Harvey from Swaab Attorneys reached out to Danielle from DR Care Solutions on 5th March 2024 in relation to her long-standing client, Mimi Prins and the need for support to determine next steps in Mimi's ageing care journey. We communicated with Angela throughout 2024 and on 13th December 2024, Angela requested that we become engaged to find quality care at home for Mimi to allow her to remain there for the rest of her life. She is currently bed-bound and has two private carers who are lovely, however not qualified to care for Mimi's care needs for the long term. Danielle spoke with Mimi's primary carer, Tracey Mannix on Friday 13th December 2024 to obtain initial background information. We arranged an urgent assessment of Mimi at her home in Castle Hill on Saturday 14th December 2024. We have been asked to arrange 24+ hour care for Mimi which needs to be in place by Monday 6th January 2024. Tracey and another private carer, 'Annie' will support Mimi and her new care team through the transition. The new care team will be set up by an Aged Care approved provider with skilled carers who are employed and fully insured to care for Mimi. Mimi was awake, alert and welcoming. She was lying in her hospital bed in the front room of her very large home in Castle Hill. She appeared comfortable and settled. She understood the need for Danielle to be there. Additionally, we had Catherine Lezer on Facetime from Perth WA. Catherine's father is Mimi's cousin. Catherine is also joint Enduring Guardian with Tracey Mannix (Primary Carer). Much of the information gathered for this life plan™ was provided by Mimi herself, Catherine and Tracey.
• Physical health including mobility, continence, • Cognitive health • Sensory (sight, hearing, oral, feet) • Activities of daily living • Emotional & mental health • Risk assessment of the home and modification recommendations • Medications, allergies & vaccinations • External assistance - including Allied Health • Social background, networks and Relationships	

• Likes & dislikes • Cultural linguistic and Spiritual needs • Estate Plan/ Advice and Life Ready Register • Financial Plan/ Advice • My Aged Care • Care and support considerations	Mimi suffers from severe Rheumatoid Arthritis and is currently bedbound. She was diagnosed with the condition at the age of 14 years. She weighs under 50 kilograms. She is right hand dominant and can write. She has had two hip replacements, two knee replacements and a left shoulder replacement. Additionally, Mimi was diagnosed with Vascular Dementia on 4th March this year. Mimi has had a couple of falls earlier this year and has been in and out of hospital mainly in relation to her Rheumatoid Arthritis and ulcers on her feet (Feb 2024). She's no longer driving however her car, and her deceased father Piet's car are still in the garage. Tracey has been tasked to sell both vehicles. **Current health, cognition and physical (mobility) condition;** Mimi is bedbound. The bed is a hospital style bed with an air mattress. The bed can move up and down depending on the carer's height and it can raise Mimi to an almost seated position. Her feet can be raised as well. She hardly moves from the bed unless she's being showered or toileted. She suffers quite a bit of pain although this new bed has helped tremendously with pain management. Showering is only completed every second day (bed baths are undertaken every other day). There is always a second carer to assist with transfers and showering. Currently, this process is problematic given that there is no full bathroom on the ground level of the house. Mimi is being showered on a commode / shower chair in the laundry area. Tracey recently arranged a hose, and handheld shower rose to be attached to the laundry sink for showering Mimi. It is not ideal. There is access to water on the other side of the wall where there is a single toilet and basin. Tracey (and Angela) are investigating an urgent modification from a plumber to enable a proper shower head to be installed. This will enable the care team to shower Mimi with ease (and dignity). Mimi has a fear of falling and therefore is not keen to be showered daily at this time. She doesn't want to be placed into a wheelchair however a 'princess chair' might be a good investment to allow her to have a change of scenery. Her bed is set up in a very good position in the front room of the house. She can see what's happening in the street, in the house and out in her backyard. She spends much of her day sleeping or reading. Mimi can pull herself up using her arms. She needs to continue to build her arm strength as best she can (we suggest purchasing 1kg weights for regular bicep work). Mimi currently assists with her transfers.

	Clinical needs: Mimi is wearing pads for urinary incontinence (not pull up pants). UTI's occur from time to time and antibiotics are arranged to treat these 'as needed'. Mimi suffers from constipation. She often doesn't have a bowel movement for up to 14 days. This needs to be sorted using diet and laxatives. We understand that she's already using laxatives without success. We suggested trying Osmolax and Prune Juice together as it has assisted other clients. Mimi previously had a small pressure sore on her sacrum which has healed nicely. All her skin is currently intact although she mentioned she gets very itchy and dry. Pressure areas to be checked regularly and her skin needs moisturiser rubbed in. If any wounds are found, they need to be treated quickly to avoid skin breakdown and the development of infection. **Sensory needs:** Mimi has great vision and doesn't require glasses. Likewise with her hearing, she has no aids and doesn't need them. She has all her own teeth with a few fillings only. Mimi requires a podiatrist to visit her at home to maintain her feet and toenail health. **The GP should be able to arrange 5 free visits per annum through the GP Health Plan.** Tracey advised she would phone to see if this can be arranged. Alternatively, a local professionally trained podiatrist (fee for service via Mimi's health fund) could be arranged. Tracey advised that she cleans and trims Mimi's fingernails. Tracey had completed Mimi's left hand however her right hand needed the nails cleaned and trimmed. Mimi is a small lady, and her hands are tiny as well so diligence is required when cleaning and cutting her nails. **Activities of daily living;** Mimi needs full assistance with all activities of daily living including showering, drying, dressing, light massage with creams to maintain her skin moisture, toileting (bowel and bladder care) and grooming (ear, teeth, hair and nail care). She can feed herself. Additionally, Mimi requires assistance and support with cleaning and maintenance of the home and garden. The home is very large and over two storeys. It is clear from clutter and extremely clean. The carers have maintained the home very well even though it's large and many rooms are not used. The current carers (1 carer per 24-hour shift) sleep on a single bed in the next room to where Mimi is located. If the overnight carers need to be in a single room upstairs, a 'baby monitor' will be required. Currently, the carers can hear when Mimi calls out to them. The carers use the upstairs bathroom for showering.

	Tracey has cleared and sorted the entire garden out which apparently was overgrown and untouched for a long time. Tracey is happy to continue to maintain the gardens once she is no longer caring for Mimi. The gardens are well manicured. Tracey was about to mow the lawns when Danielle arrived for the assessment. A standard day for Mimi is that she wakes between 6.30am and 8am and goes to sleep between 8.30pm and 10.30pm or 11pm. Some nights she has difficulties sleeping and will request a 'warm milk'. We suggested to Tracey and Mimi that they investigate Melatonin (A natural sleep aid). Mimi sleeps a lot during the day and is often lying awake at night. It might be wise to get her circadian rhythm working more as it should ie: awake during the day and sleeping at night. **Emotional and mental health;** Mimi advised there is no point wallowing about her condition or her situation. She feels she is upbeat and not suffering from anxiety nor depression. She does become frustrated at times and articulates how she is feeling. She needs strong carers who can banter with her. She has a good sense of humour. **Risk assessment of the home and modification recommendations:** the home is clear of clutter. It's very large with four bedrooms upstairs and two bathrooms upstairs. There is no internal lift, only stairs. One bedroom is an office. There are formal lounge and dining rooms. Everything is clean and the place is opened, curtains and doors with fresh air flowing in. There is a single toilet and basin on the ground floor which is attached to the laundry. The only modification required is to the laundry area (as outlined above) to provide a proper showerhead for Mimi and the carers to use. **Medications, Allergies and vaccinations:** ***Mimi is allergic to morphine and pethidine.*** She's fully vaccinated and believes the Covid vaccinations accelerated her Rheumatoid Arthritis symptoms and deterioration. Mimi is on limited medications, and they are not in a blister pack nor a Webster Pak®. They are in their original jars and boxes. • Methoblastin (to treat Rheumatoid Arthritis and Psoriasis) is taken and only on a Saturday (10mg x 1 and 2.5mg x 2) • Zyrtec (an Antihistamine) is taken 'as required'. • Norspan slow-release patches (for severe pain management – an opioid analgesic) changed once a week when Mimi has a shower.

Supplements:

- Coloxyl (for constipation)
- Vitamin D
- Vitamin C
- Folate
- Ferrograd (Iron and Vitamin C)
- Herbal teas to assist with constipation.

We suggest that Mimi's medications and supplements are placed into a Webster-Pak® arranged by her pharmacy on a fortnightly or monthly basis. This assists clients to avoid over or under medicating which often happens to older Australians. We have a preferred partnership with Webstercare®. You can find a Pharmacy location where Webster-Paks® are supplied. We checked the postcode of 2154 and can see ***My Chemist in Castle Towers and Castle Mall Pharmacy*** both use Webster-Paks®.

External assistance: The only assistance that Mimi receives is her 2 carers, Tracey and Annie. Neither are qualified to provide care however they've been doing a remarkable job in keeping Mimi clean, safe and well cared for. They are private carers and not insured, therefore it's a risk to them and to Mimi.

Gardeners and cleaners are not engaged as the carers are currently undertaking all these tasks. The carers will be expected to keep the place clean and tidy. Tracey has advised that she loves gardening (and does a superb job) and will continue to do the gardening for Mimi.

Social background, networks and relationships; Mimi was born in Holland. She is an only child. At the age of 6 years (after World War 2) Mimi and her parents came to Australia. She had an Aunty in Australia who encouraged the family to migrate. They lived in Queenscliff on Sydney's Northern Beaches. She went to the local public school and attended Wenona for her high school years.

After leaving school, Mimi went to Mitchell College for Higher Education (possibly part of Charles Sturt University) and became a fully certified chartered accountant, specialising in taxation and international taxation transactions. She worked for many years at Arthur Anderson which became PwC (Price Waterhouse Coopers). She also worked for herself. Mimi never married nor had children.

Mimi only retired around 3 years ago. She's travelled each year to the South of France (Cap Ferrat) and enjoyed travelling annually to London, Paris and went on cruises. Mimi used to love to swim in pools particularly when she travelled. It was likely it gave her pain relief.

	Even though Mimi's body was failing her, she was determined to continue to live her best life. All her trips in 2023 were cancelled due to her declining health. Mimi has friends all around the world. She's found it difficult to maintain relationships with them, particularly in recent times. She has very limited social interaction apart from her carers and others in her network. Catherine is regularly in contact with Mimi via mobile telephone using face time. Catherine was in Sydney for 22 years before relocating to Perth. **Likes and dislikes:** Mimi loves classical music. She used to paint using water colours. She loved art galleries and the Arts. Mimi loves beautiful gardens and has lovely gardens currently with roses and hydrangeas. Tracey has been taking photos of the garden and bringing them in to show Mimi. Mimi used to be a social smoker however hasn't smoked for years. She would occasionally have a glass of champagne however never was a big drinker. Mimi is a good eater however she's very small. She likes vegetarian foods and meat. She loves smoked salmon, olives, cheese, fruit and vegetables. She drinks mainly juice (poppers). She is swallowing without difficulty. Her food it cut into bite sized pieces, and she can feed herself with a spoon or fork. She drinks quite a bit of water. She's not a big, sweet eater. She loves Vitawheat biscuits and cheese for lunch. Mimi has Stan and Netflix on her television. She enjoys watching detective stories, mysteries and crime shows. The television generally doesn't go on until 6pm when the news is on. She has an iPad and enjoys reading physical books. Mimi enjoys having lotion rubbed into her skin with light massage. Her skin is very sensitive. Mimi doesn't suffer fools and can be frank with people. **Cultural, linguistic and spiritual needs;** Mimi can speak four languages fluently; English, Dutch, French and Italian. She has no religious nor spiritual requirements currently. **Personal alarms and monitoring;** While Mimi has 24-hour care in place, she doesn't require a personal alarm however if carers are upstairs, we suggest the purchase of a baby monitor near Mimi so they can keep an eye on her if they are out of her sight.

	Estate planning; We have received e-copies of Mimi's Estate Planning documents. Angela Harvey (her lawyer) and Tracey Mannix (her primary carer) are joint Powers of Attorney. Catherine Lezer (her cousin) and Tracey Mannix (her primary carer) are joint Enduring Guardians. These documents have recently been reviewed. Angela Harvey has been Mimi Prins lawyer for many years. **Advanced Care Directive:** Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask Mimi's GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner **Funeral arrangements:** We work very closely with Funeral Directors (from the Family Business Sector) and would be more than happy to introduce you to businesses that cover your area if requested. Be mindful of payment plans when it comes to funerals, as often they cost more than a standard funeral. Preplanning eases your family's burden during an emotionally difficult time. If you have specific questions, feel free to ask. **Financial planning;** Mimi is a self-funded retiree. This doesn't mean that she can't access Government Funded Services. Some services are not means tested (Commonwealth Home Support Program which is entry level access to services within My Aged Care) and others are means tested (Home Care Packages, Residential respire care and permanent residential care). For more information on how the Government Support works, please review this section on our website. Government Support | DR Care Solutions. We are unsure if Mimi has a Financial Adviser or an Accountant (however, I am sure she would). Should Mimi require the services of an Aged Care Accredited Financial Adviser, Danielle can make a referral to one for advice around the Means Tested Care Fee. Alternatively, the box on the required form can be ticked to say that Mimi will pay the full means tested care fee rather than divulging all her finances to Centrelink. Many of our clients prefer to do this. **My Aged Care;** Whilst Mimi is a self-funded retiree, she is still able to obtaining Government Funding via a Home Care Package to remain at home. We understand that she has been assessed by My Aged Care and has a code for a level 4 home care package.

	We found a document on the kitchen bench with the following number listed: **2 – 150 799 536 176** which is access to a Home Care Package. There was a home care providers card on the bench as well. We contacted the provider to double check that they hadn't enacted the package. They haven't enacted the package. We have also received a letter from My Aged Care that Angela Harvey (Lawyer and POA) collected from Mimi's. It appears that an extension was asked for to enact the package. We need to URGENTLY set up a Home Care Agreement **prior to the 7th January 2025.** Please note that a Home Care Package is Means Tested however it is lifetime capped. An Aged Care Accredited Financial Adviser can provide guidance on how Centrelink / Services Australia determine how much Mimi is required to pay the Government. **Care and Support considerations:** Mimi Prins requires 24 hours a day care and supervision. We suggest that an extra hour at each 12-hour shift is added to cover off lifting, transfers, showers, errands, shopping and handovers ie: 26 hours per day. Based on our many decades experience in providing support to clients in their homes, we believe that the care provision for Mimi needs to be 2 shifts per day with each shift being 13-hours per day. This includes an additional hour for hand over per day (ie: 2 carers over the 26 hours). The care currently provided, is one carer for 24-hour period. Tracey is currently doing about six to seven days straight (which is exhausting and dangerous). Annie the other private carer is picking up days before and after those days. Tracey works full time (elsewhere) and is on holidays from her full-time job whilst caring for Mimi. The carers undertake all the shopping, meal preparation, cleaning, laundry, bedmaking and all other homemaking tasks in addition to caring for Mimi. She doesn't need someone sitting with her every hour of the day. It is expected that all the tasks are completed in between checking in with Mimi and supporting her 'as needed'. Mimi can communicate her wants and wishes quite well.

Goals (what does the client wish to achieve): ● Options ● Preferences ● Ideal Outcome ● Carer Profile	Mimi expressed the wish to **remain at home** with quality care and support in place. We explained that getting a quality home care provider (with a Case Manager overseeing her care) having a supportive team around her is what needs to occur. Mimi has made it clear that she doesn't want to relocate into residential aged care. She has the financial means to remain at home with quality care in place until end of life therefore she should be allowed to do so. We have several clients around Australia who have 24-hour+ care in place with great outcomes. A Case Manager is normally allocated to the client from the Home Care Provider, and they will visit Mimi regularly to ensure there is complete oversight with the team of carers and that Mimi always has the appropriate care and support in place. A Case Manager is generally a Registered Nurse who can clinically quickly pick up any issues in relation to their condition, behaviour or clinical care needs. The rates being charged by providers 'fee for service' **range from $70 to $90 per hour** Monday to Friday for fully insured and employed carers (Minimum charge is generally 3 hours per visit). Weekends and public holidays incur surcharges. Review of the hours and Mimi's situation should be undertaken monthly by the Case Manager and relayed back to Mimi's guardians and attorneys, to ensure the right amount of care and support services are taking place for them at any given time. Government funding can be added to the mix of funding once it comes through. Home Care packages through the Government are means tested. **Carer Profile:** We envisage strong, caring, empathetic and highly skilled carers be employed to support Mimi to remain at home for the long term. Intelligent, well-travelled and educated carers, who can converse with her would be ideal. She mentioned that she likes carers with strong personalities who she can converse and banter with. Softly spoken, quiet and reserved carers won't do well with Mimi. At times, she swears and can become frustrated. Mimi says things that others might take offence at, so carers with 'thick skin' are needed. A team of 4 or more carers would be a great support to Mimi, physically, emotionally, and intellectually. Carers from the local area would be preferred and with a driver's license and a car will be essential.

	Mimi lives near Castle Towers Shopping Centre. The cross over of carers means that someone will always be with Mimi if one needs to do the shopping or run errands. **Mimi should never be left alone.** Homemaking skills would be advantageous to maintain the home including changing the bedlinen on both beds, undertaking the washing and laundry, cleaning and drying the bathroom after showering but have the necessary 'carer' skills to help Mimi in the shower, dressing, undressing, toilet, bed and with transfers. Given there will be two carers to transfer Mimi, a hoist may not be needed at this time. Assistance with shopping, carrying heavy groceries and meal preparation will be required. Carers who have clinical knowledge of someone living with Rheumatoid Arthritis and vascular dementia who can identify any changes in Mimi's condition would be helpful to relay back to their Case Manager (Generally a Registered Nurse). The carers need to give Mimi the space she needs when she needs it. Mimi can articulate when she's tired and needs time to rest.
Actions required: (for the client to achieve these goals)	1. **Arrange immediate in-home care and support**. We will obtain quotes from three quality home care providers in our preferred provider network for ***26 hours a day care, 7 days a week***. 2. Support the provider to urgently **enact the level 4 Home Care Package** before the 7th January 2025. 3. **Development of an Advanced Care Directive** with specific emphasis on healthcare and accommodation decisions. 4. **Consider a manual hoist as a lifting aid.** This would be for the carers to use. The carers would require expert training to operate the hoist correctly by an Occupational Therapist / Manual Handling person. 5. **Urgently obtain a quote from plumber** to see if a handheld shower rose can be installed on the other side of the toilet / basin in the laundry. 6. Consider setting up a **prepaid funeral** in event of one or both passing. Please advise Danielle for an introduction to one of our Strategic Partners (Family Business) if interested. 7. Purchase some **light (1kg) weights** that Mimi can use to maintain her upper body strength to continue to assist with pulling herself up and assisting with transfers.

	8. Consider **purchasing a baby monitor** for use next to Mimi, if the carer is upstairs. 9. Arrange a **professional Podiatrist** to visit the home to maintain Mimi's foot and nail health (Check with GP as may be able to obtain five free visits per annum via the GP Health Plan) or arrange.
Who will be involved: (Guardian, solicitor, financial planner/ advisers, family members)	● Mimi Prins ● Powers of Attorney and Enduring Guardians: Tracey Mannix, Catherine Lezer and Angela Harvey ● Danielle Robertson from DR Care Solutions ● Financial adviser or Accountant ● Estate Lawyer (also Angela Harvey) ● GP, Geriatrician, and any other medical specialists involved in Mimi's care
Review of Life Plan and the date the client needs to determine next steps:	Wednesday 18th December 2024

Signature (Consultant):  Date: Tuesday, 17th December 2024

This document is issued by Danielle Robertson Consulting Pty Limited ABN 44 605 748 392 trading as DR Care Solutions. This document contains proprietary information of DR Care Solutions and is intended for the exclusive use of the parties to whom it is provided by DR Care Solutions. Its content may not be modified, sold or otherwise provided, in whole or in part, to any other person or entity without DR Care Solutions prior written permission. © 2024 Danielle Robertson Consulting Pty Limited. All Rights Reserved